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Submitted electronically to http://www.regulations.gov.

Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

Re: Request for Information; Health Technology Ecosystem

Boehringer Ingelheim Pharmaceuticals, Inc., appreciates the opportunity to submit these comments describing how health technology and the broader digital medical ecosystem can improve access to healthcare services for Americans with mental health and substance use disorders.

Boehringer Ingelheim is a leading global research-driven biopharmaceutical company with extensive expertise developing innovative therapies to treat a variety of chronic and life-threatening diseases. The Company is committed to ensuring that patients have access to life-saving treatments and continue to develop breakthrough therapies and healthcare solutions in areas of unmet medical need.

The Company appreciates the efforts undertaken by the Centers for Medicare & Medicaid Services (CMS), the Assistant Secretary for Technology Policy (ASTP), and the Office of the National Coordinator for Health Information Technology (ONC) to examine how the effective and responsible proliferation of technology can empower patients to make better decisions for their health and well-being. Although broad in scope, there are several ancillary issue areas we would like to highlight in our responses that focus on interrelated topics not addressed directly including workforce, parity, coverage, and payment. We look forward to understanding how CMS and ASTP/ONC's efforts will touch upon these aspects to make more data available for all stakeholders that should lead to increased beneficiary access to effective digital capabilities.

RFI Focus on Patients and Caregivers – The Need to Address Mental Healthcare Shortage

According to recent estimates, 122 million Americans, or 37% of the population, lived in 5,833 mental health professional designated shortage areas as of March 31, 2021. It is also estimated that there is a need for an additional 6,398 mental health providers to fill these shortage gaps.[1]

The impact of provider shortages in the mental healthcare space is felt nationwide, and is especially acute in rural areas, with two-thirds of shortage areas being located in rural or partially rural parts of the country.[2] For example, Wyoming and Utah have the largest proportion of its populations living in mental health shortage areas, at 96.4% and 83.3%, respectively. Patients in areas of the country not specifically designated as a mental health shortage area still face barriers to access due to inadequate reimbursement rates, poor coverage of much-needed services, and struggles with finding appointment availability. In addition to geography-based disparities, certain at-risk populations, such as Native American or

[1] USAFacts.org. Over one-third of Americans live in areas lacking mental health professionals. 2021. Accessed from: https://usafacts.org/articles/over-one-third-of-americans-live-in-areas-lacking-mental-health-professionals/.
[2] Id.

incarcerated individuals may face barriers to receiving necessary care. Boehringer Ingelheim believes the CMS / ASTP / ONC (the "Agencies") should explore technological policy options that address shortages of mental health providers. Specifically, the Company urges the consideration of policies that incentivize more behavioral health providers, such as ensuring reimbursement parity, and allowing healthcare professionals to practice to the full extent of their license.

Reimbursement Parity

Lack of adequate reimbursement is a driving factor leading to workforce shortages. A 2020 *Health Affairs* article notes that reimbursement rates for behavioral health services are significantly lower than those for medical and surgical treatments, contributing to the lower network participation rates for behavioral health providers.[3]

A research report by Milliman found that average in-network reimbursement rates for behavioral health office visits are lower than for medical/surgical office visits, and as of 2017, primary care reimbursements were nearly 24% higher than behavioral reimbursements.[4] The Milliman report further found that in 2017, for 11 states, reimbursement rates for primary care offices were more than 50% higher than reimbursement rates for behavioral office visits. Another 13 states had reimbursement rates for primary care office visits that were between 30% and 40% higher.[5] The disparity in reimbursement rates is also seen in Federal healthcare programs, such as the Medicaid program. A recent study looking at 11 state Medicaid programs found that primary care providers are regularly reimbursed at a higher rate than psychiatrists for the same treatment codes.[6] The same study notes that despite Medicaid expansions in recent years, only 34.8% of psychiatrists were accepting new Medicaid patients in comparison with 71.3% of primary care physicians and 73.3% of physicians in other specialties due in part to low Medicaid reimbursement rates.[7]

Similar disparities were found between in-network and out-of-network utilization rates, with behavioral health visits being around 5 times more likely to be out-of-network versus primary care/medical office visits. The lack of in-network options impact patients through longer wait times for obtaining treatment and the need to choose between traveling a longer distance to see an in-network provider or paying a higher out-of-pocket cost to see an out-of-network provider closer to home. Boehringer Ingelheim believes such choices should not have to be made by patients suffering from behavioral or mental health issues.

Inadequate or low reimbursement rates and in-network versus out-of-network considerations present substantial barriers to maintaining a consistent and adequate workforce capable of meeting the needs of patients facing behavioral and mental health issues**. Boehringer Ingelheim strongly opposes the disparity in reimbursement rates for Federal programs like Medicaid and recommends the Agencies examine ways to ensure adequate payment levels for behavioral health services that take into consideration the actual costs of delivering them.**

Allowing Providers to Perform at Their Highest Ability

One issue facing the healthcare system is the inability of providers to practice to their highest ability or across state-lines. Many states, and some Federal healthcare programs, place restrictions on providers, such as nurse practitioners or physician's assistants, that prevent them from practicing unrestricted to the full extent of the licensure. Scope-of-practice restrictions create barriers to patients receiving the care they need and only serve to delay treatment even further in areas already facing provider shortages. Additionally, differences between state scope-of-practice requirements can make it difficult for a provider to move from one state to another and continue practicing according to their education and training. Finding ways to reduce scope-of-practice restrictions would increase treatment capabilities and improve outcomes.

[3] Maxwell J, Bourgoin A, Lindenfeld Z. Battling The Mental Health Crisis Among The Underserved Through State Medicaid Reforms. Health Affairs Blog. 2020. DOI: 10.1377/hblog20200205.346125.
[4] Davenport S, Gray T, Melek, S. Addiction and mental health vs. physical health: Widening disparities in network use and provider reimbursement. Milliman. 2019.
[5] Id.
[6] Mark TL, Parish W, Zarkin GA, et al. Comparison of Medicaid reimbursements for psychiatrists and primary care physicians. Psychiatr Serv. 2020; 71:947–950.
[7] Id.

Additionally, improving the ability of providers to practice in neighboring states could also ease the burden of areas facing provider shortages. Such an approach has already been undertaken by the Department of Veterans Affairs (VA). In 2020, the VA issued a regulation conforming its long-standing practice of allowing VA healthcare professionals to deliver healthcare services in a State other than the healthcare professional's state of licensure, registration, certification, or other state requirement, thereby enhancing beneficiaries' access to critical VA healthcare services.[8]

In issuing the regulation, the VA stated that the practice of allowing VA healthcare professionals to practice across state lines optimizes the VA healthcare workforce to meet the needs of all VA beneficiaries year-round. The VA noted that it is common practice within the VA healthcare system to have primary and specialty healthcare professionals routinely travel to smaller VA medical facilities or rural locations in nearby states to provide care that may be difficult to obtain or unavailable in that community.

Also, the VA noted its use of rural mobile health units that provide healthcare services to veterans who have difficulty accessing VA healthcare facilities and that the healthcare professionals who provide healthcare in these mobile units may provide services in various states where they may not hold a license, registration, or certification, or satisfy some other state requirement. The VA indicated that it is "critical that these healthcare professionals are protected from any adverse State action proposed or taken when performing these crucial services."

Allowing healthcare professionals to provide healthcare across state lines also gives VA the flexibility to hire qualified healthcare professionals from any state to meet the staffing needs of a VA healthcare facility where recruitment or retention is difficult. Permitting VA healthcare professionals to practice across state lines is an important incentive when trying to recruit for these vacancies, particularly during a pandemic, where private healthcare facilities have greater flexibility to offer more competitive pay and benefits.[9]

The Company recommends the Agencies investigate incorporating aspects of the VA system to help alleviate provider shortages by allowing healthcare professionals to practice across state lines based on the licensing of their "home" state. The Agencies should also consider how best to allow providers to practice across state lines remotely via telehealth or other appropriate means, not just in-person. Utilizing all means of providing patient care in this manner would improve options and access for patients, leading to more timely care and treatments, especially in areas facing healthcare professional shortages.

RFI Focus on Digital Health Adoption – *Increasing Integration, Coordination and Access to Care*

Use of Digital Technologies

Better integration, coordination, and access is vital in helping patients achieve better long-term outcomes as well as producing a more efficient healthcare system. Policies, such as ensuring the full capabilities of digital technologies are harnessed to the benefit of the patient, should be at the forefront of any discussion on improving coordination and access to behavioral health services.

Boehringer Ingelheim believes that digital technologies can offer exciting new ways to help patients in need, and we are continually working on new and innovative approaches to meet this goal. For example, in 2020 the Company embarked on a collaborative approach to develop and commercialize CT-155, a novel prescription digital therapeutic to aid in the treatment of schizophrenia. Schizophrenia is one of the 15 leading causes of disability worldwide, with approximately half of all patients exhibiting co-occurring mental and/or behavioral health disorders with annual direct medical costs from healthcare resource utilization for treatment-resistant schizophrenia estimated to be $34 billion per year.[10] Schizophrenia is a serious mental health condition that alters a person's perception of reality and impacts how they think, feel,

[8] 85 FR 71838 (12 Nov. 2020)
[9] Id.
[10] Kennedy JL, Altar CA, Taylor DL, Degtiar I, Hornberger JC. The social and economic burden of treatment-resistant schizophrenia: a systematic literature review. Int Clin Psychopharmacol. 2014;29(2):63–76. DOI: 10.1097/YIC.0b013e32836508e6 PubMed PMID: 23995856.

and behave, and patients diagnosed with schizophrenia can remain functionally impaired due to insufficiently treated negative symptoms. Significant negative symptoms, such as cognitive deficits and limited social functioning presented through emotional withdrawal, lack of motivation, and social isolation, impact the diagnosed individuals immeasurably, and for these, CT-155 provides a therapy where no alternative treatment exists. Treatment guidelines recommend tailored psychosocial intervention therapies; however, access to these interventions is limited. Providing a novel, digital therapeutic option has the potential to significantly increase patient access to treatment and positively impact their quality of life.

Boehringer Ingelheim recognizes the importance of finding new solutions for people living with neuropsychiatric disorders. As part of its innovative research program, Boehringer Ingelheim has developed and applied new ways to study the brain pathophysiology and to identify dysfunctions in the brain's neuronal pathways that may contribute to the symptoms of neuropsychiatric disorders. Combining digital innovations that are based on the principles of cognitive behavioral therapy with pharmacological solutions that also address symptoms at the root cause is key to making progress in clinical trials and improving the lives of patients with neuropsychiatric disorders.

In order to maximize the potential of digital health technologies and tools, policies should not interfere with the open collection and exchange of appropriate data. Digital health technologies and tools benefit from the ability to quickly adapt and improve to meet the needs of the patients and their providers as well as the plans administering the benefit. The ability to quickly adapt and improve is incumbent upon a rapid exchange of data that captures not only patient outcomes, but the experiences of the providers and plans as well. Feedback from all participants in the healthcare system is needed to continually grow and improve systems leading to better patient outcomes. Policies that hinder the exchange of data and participation will disincentivize the future development and use of digital health technologies and platforms.

Utilizing digital technologies also provides a unique opportunity to engage and treat the younger population. It has been shown that younger populations have a higher degree of access to digital devices, therefore providing an opportunity to develop scalable, targeted, and accessible digital approaches to achieving mental health for such a population. For example, in younger populations there is 90% smartphone ownership prevalence, with the majority using some form of social media.[11] **The Agencies should recognize the changing dynamic of reaching and treating younger populations and begin to explore how digital technologies could be utilized in an effective and safe manner as a means for increasing access points to diagnosis and treatment of behavioral health.**

Prescription Digital Therapeutics (PDTs)

A new cutting-edge therapeutic modality of medical device software includes Food and Drug Administration (FDA)-authorized products called prescription digital therapeutics (PDTs), which are beginning to improve the lives of Americans living with serious diseases and conditions like substance use disorder, attention-deficit/hyperactivity disorder, insomnia, major depression, visual impairment, diabetes, stroke, schizophrenia, and cancer. The Agencies should explore how to best facilitate the integration and widespread use of these consumer "software as a medical device" mobile medical applications. An area of focus for the Agencies should be reimbursement and coverage of PDTs in the Medicare and Medicaid programs under Medicare's existing benefit categories, which do not explicitly include "software as a medical device" as a payable direct practice expense.

PDTs are designed to provide evidence-based treatment in a convenient, user-friendly, and personalized format. Currently, the FDA has cleared over 20 PDTs targeting a variety of conditions, like irritable bowel syndrome, chronic lower back pain, and diabetes. [12] [13] PDTs must

[11] Uhlhaas, P., Torous, J. Digital tools for youth mental health. npj Digit. Med. 2019; 2, 104. https://doi.org/10.1038/s41746-019-0181-2.
[12] Phan P, Mitragotri S, Zhao Z. Digital therapeutics in the clinic. Bioeng Transl Med. 2023;8(4):e10536. doi:10.1002/btm2.10536.
[13] Academy of Managed Care Pharmacy (AMCP). List of Existing PDTs (Q1 2024). 2024. https://www.amcp.org/sites/default/files/2024-02/PDT_list_q1_2024.pdf.

be tested in clinical trials, reviewed, and cleared or granted by the U.S. FDA for efficacy and safety, and prescribed by a healthcare provider.[14] [15]

Mental health is a uniquely promising application for PDTs, with device-based brain-training exercises, skill-learning modules, and more being developed for a variety of conditions.[16]

- 23% of adults – nearly 60 million Americans– reported experiencing a mental illness in the past year, yet over half (56%) of people with a mental illness do not receive treatment. [17]

Despite federal efforts to expand parity in coverage, cost remains a barrier. Among adults with "fair" or "poor" mental health, one in five reported that their insurance plan did not cover a mental health service or treatment they needed in the past year, and 37% did not receive mental healthcare due to an absence of coverage.[18] The scarcity of providers is a critical issue. Over half of the U.S. population, 169 million people, were living in a federally designated mental health professional shortage area in 2023, and nationally, there is only one mental health provider for every 340 people. [17,19]

The remote nature of PDTs could be especially useful for rural populations by streamlining care, reducing in-person visits, enhancing convenience, and helping patients feel supported in between appointments. Additionally, CMS has supported burgeoning models of care delivery that address the health professional shortage. Specifically, the hospital-at-home health care model allows patients to receive hospital care in their homes, and studies show that the model lowers the cost of care by 38%, while improving the quality of care.[20] Together, the technology of PDTs and the hospital-at-home healthcare delivery model may be leveraged to improve access to hard-to-reach patient populations and furnish savings to the healthcare system.

Interactive and personalized PDTs may also make care more engaging and meaningful. Coding, coverage, and payment for digital mental health treatment remains elusive. Only two Medicare Administrative Contractors (MACs) appear to be making limited payments for a series of limited and confusing CMS-established digital mental health treatment service codes.[21] Moreover, these payments are very specific, which Medicare admitted was "a starting point" for payment of devices as supplies furnished incident to professional behavioral health services and "will be an iterative process."[22]

The vagueness of the coverage and payment policy is concerning and continues to be a source of provider confusion, opens up the potential for significant claims errors, leads to inconsistent reporting by providers. All of these impacts jeopardize patients with mental health illnesses by leading to inconsistent, costly, and inefficient healthcare.

[14] Brezing CA, Brixner DI. The Rise of Prescription Digital Therapeutics in Behavioral Health. Adv Ther. 2022;39(12):5301-5306. doi:10.1007/s12325-022-02320-0.

[15] Dang A, Arora D, Rane P. Role of digital therapeutics and the changing future of healthcare. J Family Med Prim Care. 2020;9(5):2207-2213. doi:10.4103/jfmpc.jfmpc_105_20.

[16] Liesch Fm Volgina D, Nessim C, Murphy D, Samson C. Prescription Digital Therapeutics U.S. Market Landscape for 2023: Current status of PDT commercialization and upcoming launches. Blue Matter Consulting. 2023. Accessed from: https://bluematterconsulting.com/wpcontent/uploads/2023/04/2023-Rx-Digital-Therapeutics-Landscape-Overview.pdf.

[17] Mental Health America. The State of Mental Health in America. Published 2024. https://mhanational.org/issues/state-mental-health-america.

[18] Pollitz K, Pestaina K, Montero A. et al. KFF Survey of Consumer Experiences with Health Insurance. Kaiser Family Foundation (KFF). 2023. Accessed from: https://www.kff.org/private-insurance/poll-finding/kff-survey-of-consumer-experienceswith-health-insurance/.

[19] Mental Health America. The State of Mental Health in America. 2024. Accessed from: https://mhanational.org/issues/state-mental-health-america.

[20] Bridger H. Home Hospital Model Reduces Costs by 38%, Improves Care, Study Says. Harvard Gazette. 2019. https://news.harvard.edu/gazette/story/2019/12/home-hospital-model-reduces-costs-by-38-improves-care/.

[21] Novitas. Fee Lookup. Accessed from: https://www.novitas-solutions.com/webcenter/portal/MedicareJH/FeeLookup; https://medicare.fcso.com/fee_lookup/fee_schedule.asp.

[22] https://www.federalregister.gov/d/2024-25382/p-1930.

Incentivize Collaborative Approaches to Care

A patient with complex behavioral or mental health issues may end up being treated by a provider that has not been specially trained to treat such complex issues and conditions. There are several reasons a patient might receive care from a provider without the necessary education and training to meet their treatment needs, including a shortage of necessary mental and behavioral health providers or a plan benefit design that does not adequately integrate the delivery of primary care and mental health treatment. Such situations not only impact patient health but also place undue stress on providers and the healthcare system as whole.

To address this concern, the Agencies (notably CMS) should focus on polices that not only address provider shortages, but also incentivize the integration of primary services with mental and behavioral health services. This includes models that promote a collaborative approach to patient care, such as the Collaborative Care Model, advanced by the American Psychiatric Association (APA) and the Academy of Psychosomatic Medicine (APM). The Collaborative Care Model is comprised of four essential elements that include the provision of care that is 1) team-driven; 2) population-focused; 3) measurement-guided; and 4) evidence-based.[23] Under a Collaborative Care Model, a multidisciplinary group of healthcare delivery professionals provide care in a coordinated fashion, and the model empowers these professionals to work at the top of their training.[24] This model is often led by a primary care provider (PCP), working with a case manager, in coordination with mental and behavioral health specialists, such as psychiatrists, psychiatric nurse practitioners, social workers, licensed counselors or psychologists. The specialist works with the PCP and provides recommendations or direct treatment for patients with complex issues.

In the report on the Collaborative Care Model, the APA and APM reviewed various studies and trials examining collaborative approaches to treatment and found that "Collaborative Care has demonstrated cost-effectiveness, significant improvements in clinical outcomes, and high levels of satisfaction in providers and patients in diverse community settings."[25] The Collaborative Care Model also increases the capacity potential to treat behavioral and mental health conditions. The APA/APM reports notes the following:

> "rather than requiring a patient to attend specialty behavioral health appointments and perpetual co-management, the goal is to make sure the patient gets what he/she needs regardless of which healthcare door he/she enters and to titrate the intensity of services to the degree of patient complexity and response to treatment. Patients with less complex disorders are managed peripherally as outcomes improve. The specialist eventually intensifies treatment for complex or treatment resistant cases via more direct consultation and management."[26]

As shown, policies that focus on incentivizing a collaborative approach to treating patients can not only increase coordination of care, but can also help address workforce shortages through more effective treatment plans with improved outcomes and lower overall costs.

RFI Focus on Patient Needs and Data Access and Integration – *Ensuring Parity Between Behavioral and Physical Healthcare*

Improve Access to Novel Therapies

Boehringer Ingelheim urges the Agencies to encourage access to new and innovative products by incentivizing their adoption and use in the marketplace. Mental health is a particularly challenging area for drug development. Compared to other therapeutic areas, mental health has seen little innovation and few novel mechanisms approved in the last decades. Some pharmaceutical companies have even discontinued their research and development (R&D)

[23] APA/APM. Dissemination of Integrated Care Within Adult Primary Care Settings: The Collaborative Care Model. 2016.
[24] Id.
[25] Id.
[26] Id.

efforts in this area.[27] Faster coverage determination of truly innovative treatments (e. g. novel mechanisms of action, breakthrough designation by FDA) would contribute to improved access and set incentives for more R&D. One way to incentivize more development and quicker adoption of innovative products is through the creation of a fast-track pathway to innovative, proven technologies specifically for the treatment of psychiatric disorders. CMS recently attempted to implement a similar program that would have provided faster access to new, innovative medical devices that have been designated as "breakthrough" by the FDA and have also received market authorization. The Medicare Coverage of Innovative Technology (MCIT) program would have allowed for national Medicare coverage for eligible devices on the date of FDA market authorization with the national coverage determination continuing for four years. In proposing the program, CMS stated that it would "give Medicare beneficiaries access to the newest innovations on the market…"

Under the current approval and coverage structure, coverage determinations can take, on average, 9-12 months, which leads to coverage uncertainty between the period of FDA market authorization and any coverage determination. Such uncertainty hampers beneficiary access to innovative products and could delay treatment and result in worsening health outcomes. Such barriers are especially vital to overcome when it comes to the treatment of behavioral health.

The development of a fast-track pathway for products designed to improve access to innovative technologies and improve the treatment and outcomes of patients that are in critical need of timely care. Barriers that slow down access to services and technologies only serve to prolong and worsen illness and could drive an acute behavioral health issue into a chronic, life-long battle.

Increase Behavioral Health Coverage Parity

Currently, patients suffering from behavioral or mental health issues may not be able to access necessary services as easily as they would if they were suffering from a physical illness. The lack of parity in coverage for behavioral and mental health is due, in part, to the stigma associated with these issues. That stigma, along with cultural and regional differences have led to varying benefit designs that treat behavioral and mental health services differently. However, in recent years, the stigmas associated with behavioral or mental health issues have softened, and states have taken steps to implement some form of mental health parity laws, ranging from requiring insurers to provide the same level of benefit for mental illness, serious mental illness or substance abuse as for other physical disorders and diseases, to requiring, at the very least, that an option of coverage for mental illness, serious mental illness, substance abuse or a combination thereof, be provided to the insured. The latter usually requires an additional or higher premium, which many patients might find prohibitive.

On the federal level, the Paul Wellstone and Pete Domenici Mental Health Parity and Addiction Equity Act of 2008 (MHPAEA) generally prevents group health plans and health insurance issuers that provide mental health or substance use disorder (MH/SUD) benefits from imposing less favorable benefit limitations on those benefits than on medical/surgical benefits. The MHPAEA originally applied to group health plans and group health insurance coverage and was amended by the Affordable Care Act (ACA) which required that most individual and small group health insurance plans, including plans sold on the Marketplace, cover mental health and substance use disorder services. Similarly, Medicaid Alternative Benefit Plans also must cover mental health and substance use disorder services as a part of the 10 essential health benefits.

Despite the progress made on both the federal and state level, barriers to coverage for mental health and substance use treatment still exist. Health plans still use restrictive standards to limit coverage for mental healthcare and use potentially restrictive criteria patients must meet to qualify for coverage. Often, these standards make it extremely difficult to get treatment.

The Agencies should explore options to improve access by strengthening the federal mental health parity requirements. This includes improving transparency and information for

[27] Insel TR, et. al. Innovative solutions to novel drug development in mental health. Neurosci Biobehav Rev. 2013 Dec;37(10 Pt 1):2438-44.

consumers about what services are covered and how they can access them; increasing access to data on services provided and outcomes achieved compared to services for other types of physical illnesses; or placing safeguards related to network adequacy. Increased transparency and monitoring could improve overall access and increase parity for behavioral and mental health services.

Improving Access to Behavioral Healthcare for Children and Young People

A recent study found that nearly half of all mental disorders in individuals start by the age of 14 and are usually preceded by non-specific psychosocial disturbances potentially evolving in any major mental disorder and accounting for 45% of the global burden of disease across the 0–25 age span.[28] The study further found that the needs of these patients are still largely unmet:

> "While some action has been taken to promote the implementation of services dedicated to young people, mental health needs during this critical period are still largely unmet. This urges redesigning preventive strategies in a youth-focused multidisciplinary and trans-diagnostic framework which might early modify possible psychopathological trajectories."[29]

Boehringer Ingelheim encourages the Agencies to work with stakeholders in developing actions that address the behavioral healthcare needs of children and young people through increased access to screenings and services tailored to meet their needs.

One way the Company believes this can be accomplished is through the strengthening of the Early and Periodic Screening, Diagnostic and Treatment (EPSDT) benefit. The EPSDT benefit is meant to provide comprehensive and preventive healthcare services for children under age 21, who are enrolled in Medicaid with the goal of ensuring access to appropriate preventive, dental, mental health, and developmental, and specialty services.

Under this benefit, states are required to provide comprehensive services and furnish all Medicaid coverable, appropriate, and medically necessary services needed to correct and ameliorate health conditions. EPSDT is made up of the following screening, diagnostic, and treatment services. States are also required to provide any additional healthcare services that are coverable under the Federal Medicaid program and found to be medically necessary to treat, correct, or reduce illnesses and conditions discovered regardless of whether the service is covered in a state's Medicaid plan. State Medicaid agencies are required to inform all Medicaid-eligible individuals under age 21 that EPSDT services are available, and provide or arrange for the provision of screening services for all children and arrange (directly or through referral) for corrective treatment as determined by child health screenings.[30]

The ability to screen and identify the need for behavioral health services early has numerous benefits for not only the patient through tailored treatments and improved long-term outcomes, but for the entire healthcare system through reduced disease burden and avoided future medical costs. A study in the International Journal of Mental Health Systems summarized key findings on the benefit of early screening and interventions:

> "Screening strategies and early detection interventions may allow for more effective healthcare pathways, by taking action long before health problems worsen or by preventing their onset. They also allow for a more personalized care in terms of tailoring health interventions to the specific sociodemographic and health-related risk factors as well as activating interventions specific to illness stage. In this regard, the application of clinical staging models has been suggested to improve health benefits, by addressing

[28] Colizzi M, Lasalvia A, Ruggeri M. Prevention and early intervention in youth mental health: is it time for a multidisciplinary and trans-diagnostic model for care? Int J Ment Health Syst. 2020;14:23. doi:10.1186/s13033-020-00356-9.
[29] Id.
[30] Medicaid.gov. Early and Periodic Screening Diagnostic and Treatment. Access from: https://www.medicaid.gov/medicaid/benefits/early-and-periodic-screening-diagnostic-and-treatment/index.html.

> the needs of people presenting at different stages along the continuum between health and disease Despite challenging, reformulating health services in this perspective may increase prevention and early intervention effectiveness, disease control and overall care, positively impacting on the health and well-being outcomes of a broader population. Not to be overlooked, it may potentially reduce disease burden and healthcare system costs."[31]

However, evidence shows that not all children and young people receive the required screenings that would identify a need for behavioral health services. In 2017, the U.S. Government Accountability Office (GAO) looked at the EPSDT program and found that nearly half of all Medicaid beneficiaries aged 20 and under did not receive screenings and services as recommended under the EPSDT benefit.[32] Older beneficiaries tended to have lower rates of screening. In its report, the GAO noted that CMS's oversight is limited, and beneficiaries may not be receiving appropriate EPSDT services when they need them.[33]

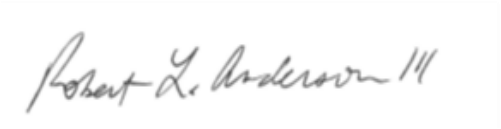
Boehringer Ingelheim encourages the Agencies to explore policies that improve access to early behavioral health screening for children and young people under the EPSDT benefit, either through increased outreach by states to ensure patients understand how to utilize their screening services and the importance of doing so, or through increasing CMS oversight of the benefit to ensure increased access and provision of screenings.

Conclusion

Boehringer Ingelheim appreciates the opportunity to provide feedback on specific aspects related to the Health Technology Ecosystem and ways to better treat behavioral health issues. The Company looks forward to working with the Agencies and respectfully requests careful consideration of the recommendations and suggestions included in these comments.

Sincerely,

SENT ELECTRONICALLY



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Boehringer Ingelheim Pharmaceuticals, Inc.

[31] Colizzi M, Lasalvia A, Ruggeri M. Prevention and early intervention in youth mental health: is it time for a multidisciplinary and trans-diagnostic model for care?. International journal of mental health systems. 2020 Dec;14:1-4.
[32] U.S. Government Accountability Office. Medicaid: Additional CMS Data and Oversight Needed to Help Ensure Children Receive Recommended Screenings. 2019. Accessed from: https://www.gao.gov/assets/gao-19-481.pdf.
[33] Id.